

Statement of Best Practice

for

Critical Illness Cover

Association of British Insurers

April 1999

Introduction

The Association of British Insurers is the trade association for insurance companies in the United Kingdom. Of its 440 members around 200 transact long-term insurance business and they account for almost 100% of the life insurance and pension business written in the United Kingdom.

This Statement of Best Practice falls under the ABI Code of Life (Non Investment) Business and covers the following:

- The description of Critical Illness Cover in Key Features
- The use of Generic Terms
- Model Wordings for Core and Additional Conditions and Model Exclusions
- The Review Process

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Appendix A: Example Key Features

General Principles

- 1.1 This Statement of Best Practice applies to critical illness cover providers who are members of the ABI and is based on the following principles.

Clarity

- 1.2 Wherever possible to aid consumer understanding, the preferred wording of Key Features and all model wordings will be in "Plain English" provided that this does not dilute or conflict with the meaning.
- 1.3 The heading (for Core and Additional Conditions and Model Exclusions) forms part of the model wording. If a provider includes an optional age limit in a particular condition, this should appear in the heading of the condition.
- 1.4 All model wordings should be as robust as possible in differentiating between what is, and is not, covered to:
 - 1.4.1 Create a clear expectation of the scope and limitations of the cover.
 - 1.4.2 Allow valid claims to be paid promptly.
 - 1.4.3 Minimise the number of disputed claims to avoid disappointment.
- 1.5 All conditions should be listed in alphabetical order (within sections as appropriate).

Generic Terminology

- 1.6 For clarity, the generic terminology should be used in all cases where the appropriate generic terms apply. Other terms to describe these features should not be used.
- 1.7 The Generic Terms are described only as far as required to establish the context. Providers may use these descriptions as definitions if they wish. Alternatively, providers may use their own definitions (provided they suit the context) or may use the terms without defining them.

Key Features

- 1.8 The Key Features requirements set out in this document are in addition to (and, in the event of a conflict, are overruled by) any regulatory, legal, 3rd Life Directive and product specific requirements for Key Features.
- 1.9 The Key Features format is intended to ensure that critical illness cover is described in the same way, regardless of the underlying product type. This is intended to allow consumers to compare the critical illness cover of different providers, accepting that the underlying products may be different.
- 1.10 All long-term products featuring critical illness cover should use the standard Key Features format – regardless of whether the sale of the product is regulated or non-regulated. This includes (but is not limited to) the following product types where critical illness cover is included as a standard feature or as an optional benefit:
 - 1.10.1 Endowment
 - 1.10.2 Whole of Life
 - 1.10.3 Term Assurance
 - 1.10.4 Stand Alone Critical Illness Cover

- 1.11 Providers should give Key Features documents to enquirers and potential customers (via intermediaries as appropriate) at the earliest opportunity to allow them to make meaningful product comparisons before purchase.

- 1.12 The format is based on the principle that critical illness cover is not usually a product in its own right, but is more frequently a benefit added to an underlying product type.

Policies with Combined Benefits

- 1.13 Where critical illness cover is offered on non-regulated product types where the Key Features do not allow the description of critical illness cover to be included (eg a policy with both income protection and critical illness cover), providers may use a separate Key Features for the critical illness cover.

Group Critical Illness Cover

- 1.14 This Statement of Best Practice applies to individual long-term critical illness contracts. Providers of group critical illness cover should follow the provisions relating to Generic Terminology and model wordings for Core and Additional Conditions and Model Exclusions.

Implementation

- 1.15 Providers may implement any measures ahead of the timetable if they wish to do so.
- 1.16 The provisions apply to new policies effected on or after the implementation date adopted by the provider. An increment or increase to an existing policy effected after that date as a new policy may be excluded if it mirrors the original contract.
- 1.17 Providers may choose to enhance cover retrospectively for customers with existing policies but are not obliged to do so.

Categorising Model Wordings

- 1.18 Model wordings for medical conditions, surgical procedures, disabilities and policy exclusions are divided into "Core Conditions", "Additional Conditions" and "Model Exclusions". The constituents will be determined at each review (and subsequently published) based on the following:
- 1.18.1 "Core Conditions" are the conditions that, together, account for the majority of critical illness claims. They will normally be any existing Core Conditions and additionally those included on at least 95% of policies on the market at the time of the review, and account for at least 1% of male or female critical illness industry claims.
- 1.18.2 "Additional Conditions" are other (non-core) conditions for which there is a model wording available. They will normally be those conditions included in the policy of at least 75% of critical illness cover policies on the market at the time of the review.
- 1.18.3 "Model Exclusions" are the policy exclusions and limitations where a model wording is available. These are generally the exclusions and limitations included in the policy of at least 50% of critical illness cover policies on the market at the time of the review.

Basis of Standard for New Model Wordings

- 1.19 All model wordings are on the basis of an "appropriate minimum standard". Providers are free to offer additional cover by including other conditions or by offering additional cover as described in Principle 1.25 below.
- 1.20 Where alternative wordings are used in the market to describe different levels of cover, the model wording will normally reflect the most robust definition, provided that this is appropriate as a minimum standard (i.e. offers meaningful cover).

- 1.21 Where several wordings are used in the market to describe essentially the same level of cover, the model wording will be based on the most robust recommended wording of the leading critical illness cover reinsurance providers (as a starting point).

Overlaps in Conditions Covered

- 1.22 Where some providers combine several conditions under one heading, others choose to offer the conditions under several headings, the model wordings will normally be based on the individual (as opposed to combined) definitions. Providers who choose to combine the wordings into a single definition would not be deemed to be using the model wording as the heading forms part of the model wording.
- 1.23 No model wording will be produced for conditions that wholly overlap with another Core or Additional Condition (e.g. no Leukaemia model wording will be available as this is wholly covered under Cancer).

Use of Model Wordings

- 1.24 Providers are free to omit any condition or exclusion and may include any other conditions or exclusions as they see fit. While providers are free to decide on the conditions and exclusions applicable to their products, where a model wording is available, it should be used.
- 1.25 Providers will be deemed to be using the model wording (for a condition or exclusion) where it is modified to provide at least equivalent cover in the following ways:
- 1.25.1 By using the model wording and showing separately the additional cover offered.
- 1.25.2 By omitting a specific limitation or exclusion in the model wording for any condition or exclusion, while leaving the remaining words unchanged. For example, this could be by omitting the words “All forms of lymphoma in the presence of any Human Immunodeficiency Virus.” from the model wording of Cancer or by omitting “off-piste skiing” from the Hazardous sports & pastimes exclusion.
- 1.25.3 By using or omitting specified optional wording (i.e. for Loss of Limbs or age limits).
- 1.26 Providers should set out their specific claim requirements in their policy, for example in a general heading along the following lines:

Example Policy Condition

All diagnoses and medical opinions must be given by a Medical Specialist who:

- holds an appointment as a Consultant at a hospital in the UK,
- is accepted by our Chief Medical Officer, and
- whose specialism is appropriate to the cause of the claim.

Reviews of Existing Model Wordings

- 1.27 No changes should be recommended to any existing agreed wordings without both the following:
- 1.27.1 A clear issue that has resulted (or is expected to result) in industry-wide problems for customers and/or providers.
- 1.27.2 Agreement (with a clear rationale) that the proposed wording change will address the issue.

Use of Key Features

General

- 2.1 Potential customers should receive Key Features at the earliest opportunity to allow them to compare products before purchase.
- 2.2 In the following guidelines, the use of *[square brackets in italics]* indicates that the wording of Key Features is illustrative of the wording that should be used and may be amended from that shown. Otherwise, the actual wording shown should be used. Providers are free to adopt any layout for their Key Features to suit their corporate style (eg 2 columns).

Front Page Headings

- 2.3 The “front-page headings” (Its Aims, Your Commitment and Risk Factors) are already mandated for regulated business. This is extended under this Statement of Best Practice to non-regulated business. Under each of these headings, the following should be used to describe these elements of the critical illness cover, in addition to the requirements for other benefits.
- 2.4 Under “Its Aims” there should be a statement describing the cover or option, depending on the context. Where benefits are payable once, on death or earlier critical illness, the wording should make it clear that only one payment is made as follows:

Its Aims

- *[To pay a guaranteed lump sum on death or earlier diagnosis of terminal illness.]*
- *[To pay you the guaranteed lump sum earlier on surviving a specified critical illness by at least x days, if you choose this cover.]*

- 2.5 Under “Your Commitment”, any obligations on the customer that are required to maintain the cover unchanged should be stated. For regular premium contracts this should include a statement about the duration of premiums. If increasing premiums are a feature of the policy, the basis should be stated. Any requirements to notify the provider of any changes in personal circumstances and the potential consequences should also be stated in this section as follows:

Your Commitment

- You agree to pay a *[regular premium for the duration of the policy]*.
- *[Your premium will increase each year as you get older.]*
- *[You must tell us about any change of occupation, smoking status or residence outside the UK - these changes could affect your cover and premium.]*

- 2.6 Under “Risk Factors” there should be a statement warning that the policy will not pay out on non-disclosure, or if the cause of a claim is related to an exclusion. If appropriate, a warning about increases in premiums should be included if premiums could increase (for example following a policy review) together with the reasons for the potential increase (such as age, claims experience, investment performance, charges, lifestyle changes as appropriate). There should be an appropriate statement to warn that cover could stop when premiums cease. These should be as follows:

Risk Factors

- We will not pay out if you do not disclose any requested information.
- *[If you stop paying your premiums your cover will end after x days.]*

- *[Your premium may increase when we review your policy after x years if, when we review our premium rates in the light of our claims experience, we apply a general increase to all policyholders. We will tell you if we apply an increase.]*
- We will not pay out if a *[critical illness / claim]* arises from an excluded cause.

Description of Critical Illness Cover

- 2.7 In a section of the Key Features, providers should show what conditions are included in the critical illness cover using the relevant sub-sections below as appropriate (conditions covered, optional conditions, child cover and further information). The following guidelines apply:
- 2.7.1 Providers should include this section in any appropriate place within the main body of the Key Features.
- 2.7.2 The opening question should refer to what is included in “critical illness” or “critical illness cover”.
- 2.7.3 The main introduction must be unambiguous that the list of conditions is an exhaustive list. Phrases such as “We cover the following:” and “The following are included:” should not be used. “The complete list of conditions we cover is as follows:” or “We cover the following conditions (and no others):” would be appropriate.
- 2.7.4 Providers should list all the conditions covered.
- 2.7.5 If a provider includes age limits in certain conditions, these should be shown in the heading of the condition.
- 2.7.6 If providers offer different levels of cover or certain extra conditions at extra cost, these should be shown separately.
- 2.7.7 Within each section, conditions should be listed in alphabetical order.
- 2.7.8 Providers may describe child critical illness cover in this section, or elsewhere, if applicable.
- 2.7.9 The “further details” should include where the full definitions and claims requirements (such as medical evidence) may be found.
- 2.7.10 The answers should be in the format shown below and following the above guidelines.

[What is included in the critical illness cover?]

- *[The complete list of conditions we cover is as follows:]*

<i>[Alzheimer's Disease before age 60</i>	<i>Loss of Limbs</i>
<i>Aorta Graft Surgery</i>	<i>Loss of Speech</i>
<i>Benign Brain Tumour</i>	<i>Major Organ Transplant</i>
<i>Blindness]</i>	<i>Motor Neurone Disease before age 60</i>
<i>Cancer</i>	<i>Multiple Sclerosis</i>
<i>[Coma</i>	<i>Paralysis / Paraplegia</i>
<i>Coronary Artery By-Pass Surgery</i>	<i>Parkinson's Disease before age 60]</i>
<i>Deafness]</i>	<i>Stroke</i>
<i>Heart Attack</i>	<i>[Terminal Illness</i>
<i>[Heart Valve Replacement or Repair</i>	<i>Third Degree Burns</i>
<i>HIV via Blood Transfusion</i>	<i>Total Permanent Disability before age 60]</i>
<i>Kidney Failure</i>	

- *[Children are also covered for the above conditions until they are 18 years old, starting from age 3 years, as long as the policy is in force. The most we will pay for a child claim is £10,000 and we will only pay one claim for each child. Claims for children do not affect your cover.]*
- *[You may also apply to be covered for {optional extra conditions}. Your personalised Key Features shows if this optional cover is included and, if so, the cost.]*
- Further details of how we will consider your claim, including the full definitions that we will use and the type of medical evidence that we will need, are contained *[in the definitions guide and policy a specimen of which is/are]* available on request.

Description of Exclusions and Limitations to Critical Illness Cover

- 2.8 In a section of the Key Features, providers should give a summary of the exclusions and limitations. This should include whichever of the following are applicable in the format shown below:
- 2.8.1 The claims notification period if exceptionally there is a specified fixed period after which a claim will not be paid (see ABI Statement of Long-Term Insurance Practice).
- 2.8.2 The survival period (normally for stand-alone critical illness cover).
- 2.8.3 The headings of any model exclusions used.
- 2.8.4 Any other limitations or exclusions which could prevent the policy paying out.
- 2.8.5 A statement regarding individual exclusions applied as part of the underwriting process.
- 2.8.6 Any restrictions that apply to child cover.
- 2.8.7 Details of where further information may be found.
- 2.9 The Key Features should show all exclusions and limitations. For model exclusions, only the headings need be shown with a reference to where the full wording may be found (as with the conditions covered).
- 2.10 The following introductory question should be included as shown. It should be shown with equal prominence to the description of the critical illness cover. Exclusions for other benefits (e.g. life cover or waiver of premium benefit) may be included under the same heading. The format of the answers should be as follows:

[What will stop the critical illness cover / policy paying out?]

- *[We will not pay a critical illness claim:]*
 - *[If you do not tell us about your claim within x months of the event.]*
 - *[On death within x days of diagnosis of the condition.]*
 - *If the cause of the claim is specifically excluded by any specific terms we apply to your [policy] when we accept your application. If applicable, we will show these [in your acceptance letter and policy schedule].*
 - *[If your claim arises within 3 months of a lapsed policy being reinstated.]*
 - *[If the cause of the claim results from aviation, criminal acts, drug abuse, failure to follow medical advice, hazardous sports and pastimes, HIV/AIDS, self-inflicted injury, war and civil commotion.]*
 - *[If the claim is for Total Permanent Disability and the cause arises while you are living abroad.]*
 - *[If the claim is for Total Permanent Disability and you changed your occupation without telling us.]*
 - *[If the claim is for child cover and the condition is familial or congenital or if symptoms first arose before the start of the cover for the child.]*
- Full details of the exclusions and limitations are contained in the *[definitions guide and policy a specimen of which is/are]* available on request.

Further Information

- 2.11 The Key Features should inform customers of the availability of the ABI Guide to Critical Illness, which may be obtained from the provider or the ABI.
- 2.12 Providers should clearly set out their complaints procedure and, where appropriate, provide contact details for the IOB or PIAOB.
- 2.13 Providers should state that their Key Features documents comply with this Statement of Best Practice.

Customer Specific Key Features

- 2.14 The Customer Specific Key Features should clearly state the basis of the premium. This should make it clear to potential customers whether the premium is level or increasing, guaranteed or reviewable (and if so, when the reviews take place). If the premium is on a level basis, this should be stated. This information should be shown prominently.

3 Generic Terms

- 3.1 When generic terms are used, ABI recommends that they should have the meanings shown and other terms should not be used in their place. This is to ensure that the terms always have the same meaning.
- 3.2 Not all the terms will apply to the critical illness cover contained in all products. Providers should only use those terms that are applicable.
- 3.3 The Generic Terms and associated descriptions as set out below are intended to establish the context in which each term should be used. Providers are free to use them as definitions, or as part definitions, or may use alternative definitions, provided these suit the context.
- 3.4 The terms “critical illness cover” and “critical illness” apply to the type of cover. Providers are free to use marketing names for their products (eg Living Plan etc) and cover (eg Living Benefit), provided that the cover is described either as “critical illness cover” or by using the words “critical illness”.

The Generic Terms

3.5 Additional Conditions

The conditions for which model wordings are available, but excluding Core Conditions and Total Permanent Disability.

3.6 Assessment Period

The period during which the permanence of a condition may be assessed before a decision on a claim will be made. A decision on whether a particular condition is expected to be permanent will normally be made within 12 months of notification of the claim provided that there is sufficient medical or other appropriate evidence to do so. The Assessment Period should only apply to claims for the conditions covered that specifically require the illness, condition or injuries to be permanent.

3.7 Core Conditions

The Core Conditions are Cancer, Coronary Artery By-Pass Surgery, Heart Attack, Kidney Failure, Major Organ Transplant, Multiple Sclerosis and Stroke.

3.8 Critical Illness Cover / Critical Illness

Includes Cancer, Heart Attack and Stroke as a minimum.

3.9 Deferred Period

The period of disability before any benefit is paid.

3.10 Irreversible

Cannot be cured by medical treatment and/or surgical procedures used by the National Health Service in the UK at the time of the claim.

3.11 Model Exclusions

The exclusions to cover for which model wordings are available.

3.12 Permanent

Expected to last throughout life, irrespective of when the cover ends or the insured person retires.

3.13 Survival Period

The period after the insured event that the insured person has to survive before a claim becomes valid. A survival period normally applies to stand-alone critical illness cover or where the amount payable on death is not the same as the amount payable for a critical illness.

4 Core & Additional Conditions

- 4.1 Providers are free to decide on the conditions applicable to their products.
- 4.2 Providers may offer additional cover whilst still being deemed to use the model wordings, subject to doing so as set out above (see General Principles).
- 4.3 The heading forms part of the model wording. However, providers may choose to include a qualifying phrase in a heading, providing this is included after the heading shown in the model wording so that the condition retains its alphabetical place in any listing. For example, this could be by replacing the heading “Cancer” with the revised heading “Cancer (most malignant types)”.
- 4.4 The use of [*square brackets and italics*] means that the wording shown may vary as follows:
- 4.4.1 The age limits for Motor Neurone Disease and Parkinson’s Disease are optional. However, if an age limit is included in the definition, it should also be included in the heading (as shown in the policy, key features and other material).
- 4.4.2 Providers may choose whether to use “elbow / knee” or “ankle / wrist” for Loss of Limbs.
- 4.4.3 The AIDS exclusion shown in the model wording of Terminal Illness should not be used in the case of accelerated cover.

The Core Conditions

4.5 Cancer

A malignant tumour characterised by the uncontrolled growth and spread of malignant cells and invasion of tissue. The term cancer includes leukaemia and Hodgkin’s disease but the following are excluded:

- All tumours which are histologically described as pre-malignant, as non-invasive or as cancer in situ.
- All forms of lymphoma in the presence of any Human Immunodeficiency Virus.
- Kaposi’s sarcoma in the presence of any Human Immunodeficiency Virus.
- Any skin cancer other than malignant melanoma.

4.6 Coronary Artery By-Pass Surgery

The undergoing of open heart surgery on the advice of a Consultant Cardiologist to correct narrowing or blockage of one or more coronary arteries with by-pass grafts but excluding balloon angioplasty, laser relief or any other procedures.

4.7 Heart Attack

The death of a portion of the heart muscle as a result of inadequate blood supply as evidenced by an episode of typical chest pain, new electrocardiograph changes and by the elevation of cardiac enzymes. The evidence must be consistent with the diagnosis of heart attack.

4.8 Kidney Failure

End stage renal failure presenting as chronic irreversible failure of both kidneys to function, as a result of which either regular renal dialysis or renal transplant is initiated.

4.9 Major Organ Transplant

The actual undergoing as a recipient of, or inclusion on an official UK waiting list for, a transplant of a heart, liver, lung, pancreas or bone marrow.

4.10 Multiple Sclerosis

A definite diagnosis by a Consultant Neurologist of Multiple Sclerosis which satisfies all of the following criteria:

- There must be current impairment of motor or sensory function, which must have persisted for a continuous period of at least six months.
- The diagnosis must be confirmed by diagnostic techniques current at the time of the claim.

4.11 Stroke

A cerebrovascular incident resulting in permanent neurological damage. Transient Ischaemic Attacks are specifically excluded.

The Additional Conditions

4.12 Aorta Graft Surgery

Undergoing surgery for disease of the aorta needing excision and surgical replacement of a portion of the diseased aorta with a graft. For this definition, aorta means the thoracic and abdominal aorta but not its branches.

4.13 Benign Brain Tumour

A non-malignant tumour in the brain resulting in permanent deficit to the neurological system. Tumours or lesions in the pituitary gland are not covered.

4.14 Blindness

Total permanent and irreversible loss of all sight in both eyes.

4.15 Coma

A state of unconsciousness with no reaction to external stimuli or internal needs, persisting continuously with the use of life support systems for a period of at least 96 hours and resulting in permanent neurological deficit. Coma secondary to alcohol or drug misuse is not covered.

4.16 Deafness

Total permanent and irreversible loss of all hearing in both ears.

4.17 Heart Valve Replacement or Repair

Undergoing open heart surgery from medical necessity to replace or repair one or more heart valves.

4.18 Loss of Limbs

The permanent physical severance of two or more limbs from above the *[elbow/wrist]* or *[knee/ankle]* joint.

4.19 Loss of Speech

Total permanent and irreversible loss of the ability to speak as a result of physical injury or disease.

4.20 Motor Neurone Disease *[before age x]*

Confirmation by a Consultant Neurologist of a definite diagnosis of Motor Neurone Disease *[before age x]*.

4.21 Paralysis / Paraplegia

Total irreversible loss of muscle function or sensation to the whole of any two limbs as a result of injury or disease. The disability must be permanent and supported by appropriate neurological evidence.

4.22 Parkinson's Disease *[before age x]*

Confirmation by a Consultant Neurologist of a definite diagnosis of Parkinson's Disease *[before age x]*.

Parkinson's Disease secondary to alcohol or drug misuse is not covered.

4.23 Terminal Illness

Advanced or rapidly progressing incurable illness where, in the opinion of an attending Consultant and our Chief Medical Officer, the life expectancy is no greater than 12 months. *[AIDS is specifically excluded and not covered under this definition.]*

4.24 Third Degree Burns

Third degree burns covering at least 20% of the body surface area.

5 Model Exclusions

- 5.1 Providers are free to omit any of the model exclusions and may include additional exclusions.
- 5.2 Providers may offer additional cover whilst still being deemed to use the model wordings, subject to doing so as set out above (see General Principles).
- 5.3 As with Core and Additional Conditions, the heading forms part of the model wording.
- 5.4 All exclusions and limitations (not only model exclusions) should be contained in one section in the policy (and Key Features as set out above).
- 5.5 Providers may define European Union as a list of EU Countries as at the start of the contract.
- 5.6 Providers should state which exclusions apply to which conditions in their policy (and other benefits as appropriate, e.g. waiver of premium benefit) and should use an introductory policy wording to suit their individual policy wording style (see the example below).

Example introductory wording for model exclusions

“ We will not pay a critical illness claim if it is caused directly or indirectly from any of the following:”

The Model Exclusions

- 5.7 Aviation
Taking part in any flying activity, other than as a passenger in a commercially licensed aircraft.
- 5.8 Criminal acts
Taking part in a criminal act.
- 5.9 Drug abuse
Alcohol or solvent abuse, or the taking of drugs except under the direction of a registered medical practitioner.
- 5.10 Failure to follow medical advice
Unreasonable failure to seek or follow medical advice.
- 5.11 Hazardous sports and pastimes
Taking part in (or practising for) boxing, caving, climbing, horse-racing, jet skiing, martial arts, mountaineering, off-piste skiing, pot-holing, power-boat racing, under-water diving, yacht racing or any race, trial or timed motor sport.
- 5.12 HIV/AIDS
Infection with Human Immunodeficiency Virus (HIV) or conditions due to any Acquired Immune Deficiency Syndrome (AIDS).
- 5.13 Living abroad
Living outside of the European Union for more than 13 consecutive weeks in any 12 months.
- 5.14 Self-inflicted injury

Intentional self-inflicted injury.

5.15 War and civil commotion

War, invasion, hostilities (whether war is declared or not), civil war, rebellion, revolution or taking part in a riot or civil commotion.

6 Total Permanent Disability

6.1 There are a number of definitions used for Total Permanent Disability. For example, “total disability” may be measured by assessing the person’s ability to perform certain of the following:

6.1.1 The insured person’s “ Own Occupation” .

6.1.2 “Suited Occupations”.

6.1.3 “ Any Occupation” .

6.1.4 A number of specified “ Activities of Daily Living” .

6.1.5 A number of specified “ Functional Ability Tests” .

Change of Occupation

6.2 Depending on whether the definition used relies on the occupation of the person covered, this occupation may be disclosed and underwritten at the outset of the policy. Where this applies, subsequent changes of occupation may vary the initial underwriting assessment. Providers normally deal with this issue in one of the following ways:

6.2.1 Notification is required. The new occupation is re-underwritten and the terms of the contract are adjusted if necessary. The customer’s ability to perform an occupation (or one that is suited, depending on the definition) is judged against the occupation most recently notified (if all changes have been notified as required).

6.2.2 Notification is not required. The customer’s ability to perform an occupation (or one that is suited, depending on the definition) is judged against the occupation being followed immediately before the claim. In this case, the provider accepts the risk of changes in occupation.

6.2.3 Alternatively, the provider judges the claim against the occupation declared in the application, regardless of subsequent changes.

6.3 “ Occupation” generally means a trade, profession or type of work undertaken for profit or pay. It is not a specific job with any particular employer and is independent of location.

Guidelines

6.4 Providers are free to use any one or more definitions of disability they wish, including but not limited to those shown in 6.1 above.

6.5 Providers should make it clear in Key Features which procedure is adopted in relation to change of occupation together with the potential consequences.

6.6 If providers require notification of changes in occupation, they should periodically remind the policyholder.

7 Review Process

- 7.1 This document has been prepared by following the normal process that is proposed for future reviews, following the principles above.
- 7.2 Industry-wide provisions, such as the Key Features format set out in this document, and model wordings based on market practice, legislation and medical science should be reviewed regularly to ensure that they remain up to date. The ABI will therefore make provision for reviews as follows.

Types of Review

- 7.3 There will be 2 types of review: Full Reviews (normally every 3 years) and Intermediate Reviews. The ABI Critical Illness Working Party will carry out all Full and Intermediate Reviews.

Full Reviews

- 7.4 Full Reviews will be carried out every 3 years unless the IP & CI Insurers Forum decides not to carry out the Full Review in which case it will be deferred for up to 1 year.
- 7.5 At a Full Review, the scope will be to review the Key Features format and all model wordings in line with the principles set out above. This will take into account changes in medical science, relevant "events" (such as changes in legislation since the last Full or Intermediate Review), experience, available IFA research, and current market practice. The process should use the expertise of Critical Illness Working Party members and appropriate liaisons.
- 7.6 At a Full Review, the recommendations will include a recommended process for implementing any changes and the review process itself should be reviewed and any changes put forward for the next review.

Intermediate Reviews

- 7.7 An Intermediate Review may be held where a compelling issue is raised that is, for example, of a legal or regulatory nature. Less compelling issues will normally be dealt with at a Full Review.
- 7.8 The scope of Intermediate Reviews will be limited to the agreed impact of the issue raised and recommendations for implementing any changes. Other issues will be outside the scope of the Intermediate Review.
- 7.9 The normal process for establishing an Intermediate Review will be:
 - 7.9.1 An issue is raised with the ABI through the IP & CI Insurers Forum.
 - 7.9.2 The Forum decides on whether to carry out the Intermediate Review - otherwise the issue is recorded for the next Full Review.

Example Key Features

The ABC Life Assurance Company

KEY FEATURES OF THE LEVEL TERM ASSURANCE PLAN

Its Aims

- To pay a guaranteed lump sum on death.
- To pay you the guaranteed lump sum earlier on surviving a specified critical illness, if you choose this cover.
- To provide the amount of cover you choose.
- To provide cover for the period you choose.

Your Commitment

- You agree to pay a premium by direct debit every month for the duration of the policy.
- You must tell us about any change of smoking status - this could change your premium.
- If your policy includes critical illness cover, you must also tell us about any change of occupation or residence outside the UK. These changes could affect your cover and premium.

Risk Factors

- We will not pay out if you do not disclose any requested information.
- If you stop paying your premiums your cover will end after 30 days.
- Your premium may increase when we review your policy after 10 years if, when we review our premium rates in the light of our claims experience, we apply a general increase to all policyholders. We will tell you if we apply an increase.
- We will not pay out if a critical illness arises from an excluded cause.
- The policy has no cash-in value.

How does the policy work?

- You decide which protection benefits you want. You can choose one of the following options:
 - To pay out on death only.
 - To pay out on death or earlier critical illness.
- You decide how long you want the cover for. This can be any period between 5 and 25 years.
- You pay a premium every month to keep your cover in force.

How much does the policy pay out?

- You decide how much cover you would like. The more cover and the longer the period of cover you choose, the higher your monthly premium will be.
- The policy only pays out the main benefit once, on death (or earlier critical illness if you choose this cover) and then the policy ends.
- Your personal illustration is attached. This shows you how much cover and the period of cover you have chosen, whether critical illness cover is included and the monthly cost.

What is included in the critical illness cover?

- The complete list of conditions we cover is:

Alzheimer's Disease before age 60
Aorta Graft Surgery
Benign Brain Tumour
Blindness
Cancer
Coma
Coronary Artery By-Pass Surgery
Deafness
Heart Attack
Heart Valve Replacement or Repair
HIV via Blood Transfusion
Kidney Failure
Loss of Limbs
Loss of Speech
Major Organ Transplant
Motor Neurone Disease before age 60
Multiple Sclerosis
Paralysis / Paraplegia
Parkinson's Disease before age 60
Stroke
Terminal Illness
Third Degree Burns
Total Permanent Disability before age 60

- Further details of how we will consider your claim, including the full definitions that we will use and the type of medical evidence that we will need, are contained in the policy, a specimen of which is available on request.

Can children have critical illness cover?

- If you choose critical illness cover, the children of those insured are also covered. Children are covered until they are 18 years old, starting from age 3 years, as long as the policy is in force.
- The maximum amount we will pay for a child claim is £10,000 and we will only pay one claim for each child.
- Claims for children do not affect your cover.

What will stop the policy paying out?

- We will not pay a critical illness claim:
 - If you do not tell us about your claim within 6 months of the event.
 - If the cause of the claim is specifically excluded by any specific terms we apply to your policy when we accept your application. If applicable, we will show these in your acceptance letter and policy schedule.
 - If your claim arises within 3 months of a lapsed policy being reinstated.
 - If the cause of the claim results from aviation, criminal acts, drug abuse, failure to follow medical advice, hazardous sports and pastimes, HIV/AIDS, self-inflicted injury, war and civil commotion.
 - If the claim is for cancer that is diagnosed in the first six months of the policy.
 - If the claim is for Total Permanent Disability and the cause arises while you are living abroad.
 - If the claim is for Total Permanent Disability and you changed your occupation without telling us.
 - If the claim is for child cover and the condition is familial or congenital, or if symptoms first arose before the start of the cover for the child, or if the child dies within 28 days of diagnosis of the condition.
- We will not pay out on death as a result of suicide in the first year of the policy.
- Full details of the exclusions and limitations are contained in the policy, a specimen of which is available on request.

Can the policy cover someone other than me?

- Yes. You can apply for cover for yourself or for joint cover for you and another person. Alternatively, you can apply for cover for one or two people not including you.
- If you choose joint cover, the policy only pays out once. This is when either person dies, or survives a specified critical illness (if you choose this cover), whichever is first.

What other options are available?

- You can choose to include Waiver of Premium Benefit. This optional benefit pays your premiums if you unfit to work in a suited occupation as a result of illness or injury for longer than 6 months until you are fit to return to work.
- Your personal illustration shows you whether Waiver of Premium Benefit is included and, if so, the cost. Further details are contained in the policy, a specimen of which is available on request.

FURTHER INFORMATION

Your Cancellation Rights

After we accept your proposal, we will send you a notice of your right to cancel. You will then have 14 days in which you can cancel the policy. If you do this, you will get a refund of any premiums you have paid.

Tax

Critical illness and death benefit payments are free of UK income tax and capital gains tax. On death, these payments may be subject to inheritance tax. You may be able to mitigate the inheritance tax by using an appropriate trust. Ask your Financial Adviser for details if you are interested in using a trust.

The Government may change the tax position described.

Law

The Law of England applies to this policy.

Complaints

If you have any cause for complaint about this policy, or about any aspect of our service, you should contact our Customer Service Manager at:

ABC Life, Our Road, Any Town, County AB1 2CD

Telephone: 01234 56789

If we do not deal with your complaint to your satisfaction, you may then contact the Personal Investment Authority Ombudsman Bureau at:

Hertsmere House, Hertsmere Road, London E14 4AB

Telephone: 0171 216 0016

Making a complaint will not affect your right to take legal proceedings. We will send you details of compensation arrangements on request.

The ABI Guide to Critical Illness

General information about critical illness cover is contained in the ABI (Association of British Insurers) Guide to Critical Illness. We will send you a copy on request. Alternatively, you can get a copy by writing to the ABI at 51 Gresham St, London EC2V 7HQ.

Please Note

This leaflet complies with the ABI Statement of Best Practice for Critical Illness Cover. It is a guide to the Level Term Assurance Plan and is based on our understanding of current law and Inland Revenue practice. Further details are contained in the plan schedule and standard provisions, which make up the policy. We recommend you seek specialist advice about the legal and tax information contained in this leaflet.

ABC Life is registered in England no 01234567.
The registered office is ABC Life, Our Road, Any Town, County AB1 2CD.
ABC Life is regulated by the Personal Investment Authority.

Implementation Process and Timetable

The provisions of the Statement of Best Practice will be phased-in to allow a smooth transition and to allow a reasonable time for providers to adopt them. Providers are free to implement any of the provisions earlier if they wish, but will be deemed to be following the Statement of Best Practice if they implement the proposals within the following timetable.

30 April 1999 Issue Statement of Best Practice

June 1999 Workshops & Seminars

Industry seminars/workshops to facilitate the introduction of the proposals and deal with any issues arising.

31 July 1999 Phase 1 - Key Features

Providers will either have their Key Features updated in the new format for the start of Phase 1, or will do so when the documents are next reprinted, provided this is before the end of 1999.

In addition to adopting the Key Features format during Phase 1, providers may also adopt the model wordings and generic terms in policy documents and other material. These have been finalised and available for providers to use ahead of Phase 2. It is for providers to decide if they wish to update all their material together for consistency.

30 June 2000 Phase 2 - Full Implementation

Providers should use the model wordings by either:

- Adopting the model wordings in policies and other material.
- or
- As an interim measure, use the "Claims Adjudication Basis" until 31 December 2000.

The "Claims Adjudication Basis" works as follows:

- The provider will adjudicate claims using either the policy wording or the model wording – whichever is better for the customer in the circumstances of their claim.
- To ensure the customer knows what they can claim for, the provider should give each customer a written Claims Adjudication Statement.
- The Statement should extend the scope of cover to include the appropriate model wordings, and list the applicable model wordings.
- The applicable model wordings are those for all the conditions that the provider covers. However, if the provider's policy has already adopted the model wording for a particular condition, it may be omitted from the Statement.
- The Statement should be included with Key Features and with the policy documents. An example Statement is shown below.

Example Claims Adjudication Statement

The Level Term Assurance Plan Critical Illness Cover

In the event of a claim for a critical illness under the Level Term Assurance Plan, we will extend the definition of critical illness cover as set out in section X (y) of the policy to include the following additional definitions. This leaflet is part of your policy and should be kept together with the other policy documents in a safe place.

Aorta Graft Surgery

Undergoing surgery for disease of the aorta needing excision and surgical replacement of a portion of the diseased aorta with a graft. For this definition, aorta means the thoracic and abdominal aorta but not its branches.

Benign Brain Tumour

A non-malignant tumour in the brain resulting in permanent deficit to the neurological system. Tumours or lesions in the pituitary gland are not covered.

Blindness

Total permanent loss of all sight in both eyes.

Burns

Third degree burns covering at least 20% of the body surface area.

Cancer

A malignant tumour characterised by the uncontrolled growth and spread of malignant cells and invasion of tissue. The term cancer includes leukaemia and Hodgkin's disease but the following cancers are excluded:

- all tumours which are histologically described as pre-malignant, as non-invasive or as cancer in situ,
- all forms of lymphoma in the presence of any Human Immunodeficiency Virus,
- Kaposi's sarcoma in the presence of any Human Immunodeficiency Virus and
- any skin cancer other than malignant melanoma.

Deafness

Total permanent loss of all hearing in both ears.

Heart Attack

The death of a portion of the heart muscle as a result of inadequate blood supply as evidenced by an episode of typical chest pain, new electrocardiograph changes and by the elevation of cardiac enzymes. The evidence must be consistent with the diagnosis of heart attack.

Heart Valve Replacement or Repair

Undergoing open heart surgery from medical necessity to replace or repair one or more heart valves.

Loss of Limbs

The permanent physical severance of two or more limbs from above the wrist or ankle joint.

Loss of Speech

Total permanent loss of the ability to speak as a result of physical injury or disease.

Major Organ Transplant

The actual undergoing as a recipient of, or inclusion on an official UK waiting list for, a transplant of a heart, liver, lung, pancreas or bone marrow.

Multiple Sclerosis

A definite diagnosis by a Consultant Neurologist of Multiple Sclerosis which satisfies all of the following criteria:

- There must be current impairment of motor or sensory function, which must have persisted for a continuous period of at least six months.
- The diagnosis must be confirmed by diagnostic techniques current at the time of the claim.

Motor Neurone Disease before age 60

Confirmation by a Consultant Neurologist of a definite diagnosis of Motor Neurone Disease before age 60.

Paralysis / Paraplegia

Total loss of muscle function or sensation to the whole of any two limbs as a result of injury or disease. The disability must be permanent and supported by appropriate neurological evidence.

Parkinson's Disease before age 60

A definite diagnosis of Parkinson's Disease made by a Consultant Neurologist before age 60. Parkinson's Disease secondary to alcohol or drug misuse is not covered.

Terminal Illness

Advanced or rapidly progressing incurable illness, where in the opinion of an attending Consultant and our Chief Medical Officer, the life expectancy is no greater than 12 months.

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